# Falls

Use this guideline to help determine which patients following a fall require referral to a medical facility and if so, to which type of facility. Use the **'transient loss of consciousness'** guideline if the patient is suspected to have fallen due to loss of consciousness.

- Assess the patient, including an assessment of the features contained within the flag table.
- In addition, perform a falls risk assessment if the patient is aged greater than or equal to 65 years, is living independently, and is not being transported to an ED by ambulance. Gain consent and refer the patient to a falls prevention service if any abnormalities are found. Alternatively, refer the patient to their GP if a falls referral pathway is not available.

## Red flags

**Should be seen in ED, usually via ambulance**

- Clinically significant injury.
- Clinically significant pain.
- Abnormal vital signs.
- Signs of stroke.
- Headache with features of concern.
- New onset of visual disturbance.
- Unable to mobilise when normally able to do so.
- Unstable medical condition contributing to the fall.

## Orange flags

**Should be followed up in primary care**

# Falls

- More than one fall in the last week.
- Postural hypotension.
- Recent change in medication.
- Minor injury requiring non-urgent treatment.
- New reduction in mobility but able to weight bear.

## Referral and transport

- If one or more red flags are present the patient must be given a clear recommendation to be assessed by a doctor within two hours and should usually be given a clear recommendation to be transported to ED, usually by ambulance.
- If the patient has one or more red flags and is being referred to primary care:
  - The patient's anticipated clinical needs must be able to be safely met in primary care, and
  - A clinician within primary care must be contacted directly by ambulance personnel who agrees to accept the patient, and
  - Safe transport (if required) must be available.
- If any orange flags are present (and no red flags) the patient should be given a clear recommendation to be followed up in primary care.
- If no red and no orange flags are present, the patient is usually suitable to be given a clear recommendation to remain in the community with self-care. Advise the patient to be seen in primary care or seek advice via Healthline on 0800611 116 if they have any concerns.

## Additional information

### General principles

- A patient that has fallen without a clear mechanical cause, for example a trip or slip, requires a thorough history and clinical examination to rule out a cause for collapse.
- A patient that has fallen always requires an assessment to rule out injury, even if they appear to be uninjured.
- Do not use the term 'lift assist' when referring to a patient that has fallen as this term should only be used when a crew is assisting another crew to lift a patient.

# Falls

- Falling is not a natural part of aging and all patients aged greater than or equal to 65 years who have fallen require an assessment that considers referral to a falls prevention service.
- Have a raised index for suspicion of injury if the patient:
  - Has fallen a significant height, for example greater than one metre or five stairs in an adult, or
  - Is taking an anticoagulant, dual antiplatelet therapy, or has a known bleeding disorder.
- Always consider the patient's comorbidities and social circumstances.
- Determining that a headache is associated with features of concern requires clinical judgement. Examples of features of concern include:
  - Features within the red flags of the headache guideline.
  - New onset of headache following the fall, especially if worsening, or associated with nausea and/or vomiting.
  - Taking an anticoagulant or dual antiplatelet therapy.
- Examples of unstable medical conditions contributing to the fall include diabetes with poor glucose control and poorly controlled Parkinson's disease. Clinical judgement is required when determining that a medical condition requires review in an ED or review in primary care.
- Postural hypotension is present if there is a fall of greater than 20 mmHg in the systolic BP or greater than 10 mmHg in the diastolic BP when standing.

## Falls risk assessment

- Falls are a common cause of injury and loss of independence in older patients.
- Ambulance personnel have an important role in referring older patients at risk of a fall to a falls referral pathway, as this reduces the risk of further falls and injury.
- Referral to a falls referral pathway is not required if the patient is living in an aged residential care facility because the facility personnel are required to manage this. Patients living in an independent unit (for example a villa) in the same complex as an aged residential care facility are designated as living independently.
- Ask the patient the following questions:
  - Have you slipped, tripped or fallen in the last year?
  - Do you need to use your hands to get out of a chair?
  - Are there any activities you've stopped doing because you are afraid of falling?
- Refer the patient to a falls referral pathway if:
  - The patient answered 'yes' to any of the questions, or

# Falls

- Personnel consider the patient is at risk of falling.

- Examine the environment for hazards which may contribute to the risk of falling. Examples include rugs, mats, cords and poor footwear. Eliminate these hazards with the patient's permission if feasible.

## Referring the patient to a falls prevention service

- Falls prevention services offer follow up and support, which may include:
  - Access to strength and balance programmes.
  - The provision of mobility aids.
  - Vision testing.
  - Medication review.

- To refer a patient to a falls prevention service via ePRF:
  - HHStJ personnel: type 'referral@falls' in the Disposition section of the ePRF.
  - WFA personnel: use the 'note to GP' function in ePRF.

- When making a referral, include as much information in the ePRF as possible to enable the falls prevention service to offer the patient the most appropriate support. For example:
  - The patient's contact details (especially their phone number).
  - The answers to the falls risk questions.
  - Whether the patient uses a mobility aid.
  - Whether the patient requires assistance with their activities of daily living, for example showering.
  - Whether the patient can leave their home unassisted.
  - Any obvious falls hazards in the patient's home.

## More Information

### Version history

**9 June 2025** 1.0.5.3
**Update:** Guideline redeveloped.